SACTCHECK · NCCP 00890

# My complete treatment handbook

## Fruquintinib (Fruzaqla) monotherapy

All information can be read on paper. Keep this with your treatment record.

> Use the urgent instructions and contact numbers in the following companion pages. Example telephone numbers must be replaced by your team before patient use. Your individual treatment plan takes precedence.

## Contents


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2. Full treatment reference: medicines, schedule, tests, symptoms and follow-up — page 5
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SACTCheck v0.84.35 · Clinical, pharmacy and patient review pending. Formal consent is supplied separately.

# Fruquintinib (Fruzaqla) monotherapy

Targeted capsule treatment to help control metastatic bowel cancer after previous therapies.

**Metastatic colorectal cancer after standard therapies and trifluridine/tipiracil or regorafenib** · NCCP 00890 v2a · Document revised 5 November 2026

**Your personal plan:** this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Previously treated metastatic colorectal cancer; follow the individual prescription and monitoring plan.

## My overall treatment

Days 1–21: prescribed dose once daily, with or without food. Days 22–28: no fruquintinib.

**28-day cycle**
Repeat only when authorised. No fixed final cycle; the team reviews benefit and side effects.

## A typical 28-day cycle

| Day 1–7 | Day 8–21 | Day 22–28 |
|---|---|---|
| Start with a clear calendar | Continue capsules and monitoring | Capsule-free week |

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

## My tests and checks

| When | What is checked | Timing / reason |
|---|---|---|
| **Before** | **Baseline checks**<br>Required | Blood count, kidneys/liver, thyroid, urine protein and controlled blood pressure. |
| **Early / later** | **Blood pressure**<br>Required | Weekly for the first month, then monthly or more often if needed. |
| **During** | **Bloods and urine**<br>Required / conditional | Regular blood count, kidney/liver, thyroid and urine tests. Warfarin needs extra INR checks. |
| **Review / after** | **Response and recovery**<br>Individual plan | Scans, markers and follow-up on your consultant’s plan; review unresolved effects. |

## Between treatments

- Report painful hands/feet, bleeding, bowel changes or poor intake early, including during the rest week.
- Do not replace a vomited dose. Follow the missed-dose plan; if exactly 12 hours late or unsure, call the team.

Confirm capsule strength, calendar, skin care, test dates and urgent contacts. Swallow capsules whole; do not open them.

**When to get help**

**Call 112 / 999** for severe breathing difficulty, severe chest pain, collapse or a seizure.

**Call oncology immediately:** temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital’s temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

# What to watch for

You do not need to know which medicine caused a symptom before calling.

### 1 Head / thyroid / energy

**Fruquintinib**

Fatigue and thyroid changes can affect daily life. Severe headache, confusion, seizure or visual change needs emergency help.

### 3 Heart / blood pressure

**Fruquintinib**

Blood pressure may rise without symptoms. Keep scheduled checks. Chest pain, collapse or severe breathlessness needs emergency care.

### 5 Liver

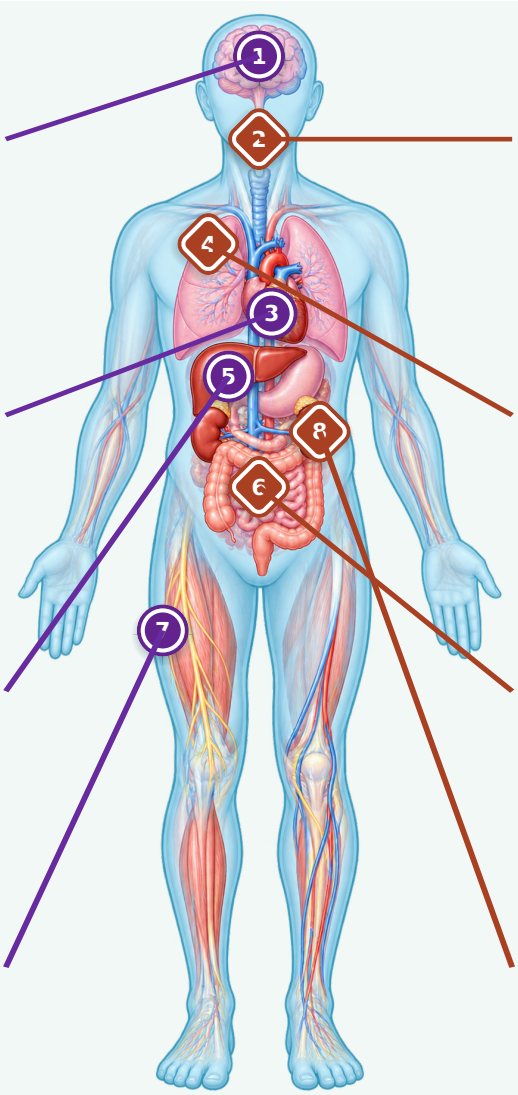
**Fruquintinib**

Liver tests can change before symptoms occur. Report yellow eyes or skin, dark urine or persistent abdominal pain.

### 7 Hands / feet / skin

**Fruquintinib**

Pain, redness, swelling or blisters can affect walking or using your hands. Report early and follow the skin-care plan.




### 2 Mouth / appetite

**Fruquintinib**

Mouth soreness, voice change and poor appetite can affect eating. Report difficulty drinking or weight loss.

### 4 Blood / bleeding

**Fruquintinib**

New bleeding, easy bruising, fever or sudden illness needs prompt advice. Heavy bleeding needs emergency help.

### 6 Bowel / abdomen

**Fruquintinib**

Diarrhoea can cause dehydration. Severe persistent pain, black stools or blood in stools needs urgent help.

### 8 Kidneys / urine

**Fruquintinib**

Protein in urine may have no early symptoms. Attend urine checks and report swelling or much less urine.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Get urgent help for severe or persistent tummy pain, black or bloody stools, coughing or vomiting blood, new severe headache, confusion, seizure, visual change, chest pain, sudden breathlessness or very little urine. Report fever, sudden illness, painful hands/feet and new bleeding promptly.

**When to get help**

**Call 112 / 999** for severe breathing difficulty, severe chest pain, collapse or a seizure.

**Call oncology immediately:** temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

# Keep your treatment details to hand

**Metastatic colorectal cancer after standard therapies and trifluridine/tipiracil or regorafenib** · NCCP 00890 v2a · Document revised 5 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

**FRONT / CONTACTS**

## My Treatment Card

SACTCheck

Name: _____

**Fruquintinib (Fruzaqla)**

28 days · capsules Days 1–21 · no capsules Days 22–28.

Phase: _____ Cycle plan: team to confirm

**Daytime (EXAMPLE)** 087 12345678

**Out of hours (EXAMPLE)** 087 87654321

**Unwell or fever? Call oncology now.**
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

CUT outer border

**BACK / TREATMENT RECORD**

## My cycle record

Rolling record — follow your agreed treatment plan.

| Cycle | Date / initials |
|---|---|
| | |
| | |
| | |
| | |

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

**FOLD centre — do not cut**

## Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

## Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

### iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

### Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

## Your local contacts and resources

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

NCCP 00890 · VERSION 2A

# Your treatment, explained

**Metastatic colorectal cancer after standard therapies and trifluridine/tipiracil or regorafenib** · NCCP 00890 v2a · Document revised 5 November 2026

**Your personal plan:** this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Fruquintinib (Fruzaqla) monotherapy

28-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

## When to get help

### Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

### Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Get urgent help for severe or persistent tummy pain, black or bloody stools, coughing or vomiting blood, new severe headache, confusion, seizure, visual change, chest pain, sudden breathlessness or very little urine. Report fever, sudden illness, painful hands/feet and new bleeding promptly.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: smpc

**Choose a topic to read**

YOUR COMPLETE REFERENCE

# Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.


- How each drug works
- Capsules and rest days
- Your 28-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- Why this treatment may be offered
- How fruquintinib works
- Your capsule and rest-day plan
- Checks and follow-up
- Skin, mouth, eating and energy
- Symptoms needing urgent help
- Other medicines, wounds and procedures
- Pregnancy precautions and safe storage
- Supportive medicines
- Understanding the evidence
- Questions to ask

Sources

UNDERSTAND YOUR TREATMENT

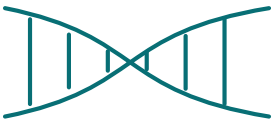
# Your targeted medicine

Fruquintinib is a targeted capsule treatment to help control bowel cancer that has spread after previous treatments. The team checks which treatments you have received, whether they remain suitable, and your priorities.

TARGETED THERAPY CAPSULE

## Fruquintinib

Fruzaqla



**Blocks VEGF receptor signals**

Fruquintinib blocks VEGF receptor signals that help tumours form blood vessels. Reducing these signals can slow cancer growth. Blood vessels in healthy tissues are also affected, which helps explain blood-pressure, bleeding, protein-in-urine and wound-healing risks.

VEGF signals → Receptors blocked → Blood-vessel growth reduced

**What to watch for**

Report painful hands/feet, bowel changes, bleeding or neurological symptoms; attend blood-pressure, urine and blood checks even if well.

Sources: smpc

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

**Explore your existing anatomy and side-effect map**

# Your anatomical side-effect map

## A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

### 1 Head / thyroid / energy

**Fruquintinib**

Fatigue and thyroid changes can affect daily life. Severe headache, confusion, seizure or visual change needs emergency help.

### 3 Heart / blood pressure

**Fruquintinib**

Blood pressure may rise without symptoms. Keep scheduled checks. Chest pain, collapse or severe breathlessness needs emergency care.

### 5 Liver

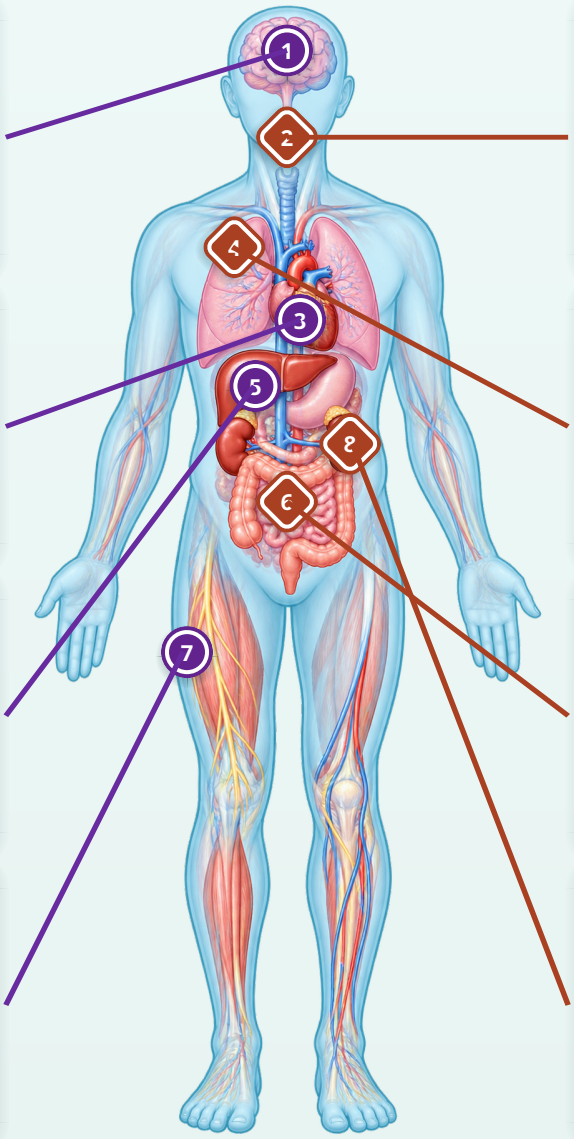
**Fruquintinib**

Liver tests can change before symptoms occur. Report yellow eyes or skin, dark urine or persistent abdominal pain.

### 7 Hands / feet / skin

**Fruquintinib**

Pain, redness, swelling or blisters can affect walking or using your hands. Report early and follow the skin-care plan.




### 2 Mouth / appetite

**Fruquintinib**

Mouth soreness, voice change and poor appetite can affect eating. Report difficulty drinking or weight loss.

### 4 Blood / bleeding

**Fruquintinib**

New bleeding, easy bruising, fever or sudden illness needs prompt advice. Heavy bleeding needs emergency help.

### 6 Bowel / abdomen

**Fruquintinib**

Diarrhoea can cause dehydration. Severe persistent pain, black stools or blood in stools needs urgent help.

### 8 Kidneys / urine

**Fruquintinib**

Protein in urine may have no early symptoms. Attend urine checks and report swelling or much less urine.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

# Capsules and rest days

28-DAY CYCLE

### 21 capsule days, then seven rest days

D1–21 D22–28

Take the prescribed fruquintinib dose once daily on Days 1–21, then take no fruquintinib on Days 22–28. Take it at about the same time each day, with or without food. Repeat only when your team authorises the next cycle.

No fixed final cycle. Continue only while benefit and tolerability justify treatment.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

# Tests before, during and after treatment

NCCP specifies regular blood/urine/thyroid tests but no universal frequency for each. Ask your team to write the dates. Scans and tumour markers are individualised.

| Check | Before starting | First month | Later cycles | A hold or procedure | After treatment |
|---|---|---|---|---|---|
| **Blood pressure** | Control existing hypertension | Weekly blood-pressure checks | Monthly and as clinically indicated | Review control before restarting | Follow persistent hypertension as agreed |
| **Blood count, kidneys, liver and thyroid** | Baseline tests | Regular tests on the team plan | Continue regular tests | Repeat abnormal tests as directed | Follow unresolved effects |
| **Urine protein** | Baseline urine dipstick | Regular urine checks | Extra measurement if protein is detected | Review kidney/urine findings before restarting | Individual follow-up if abnormal |
| **INR if taking warfarin** | Review anticoagulation | Weekly INR until a stable dose | Then before each cycle | Additional checks when medicines or health change | Anticoagulation-team plan |
| **Scans and clinical review** | Confirm previous treatments and goals | Review symptoms, skin, nutrition and adherence | Disease assessment as directed by consultant | Team authorises any restart | Agree subsequent care and contacts |

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00890 v2a official fruquintinib regimen

THE WHOLE TREATMENT JOURNEY

# Fruquintinib (Fruzaqla) monotherapy: treatment and checks

## Metastatic colorectal cancer after standard therapies and trifluridine/tipiracil or regorafenib

Take the prescribed fruquintinib dose once daily on Days 1–21, then take no fruquintinib on Days 22–28. Take it at about the same time each day, with or without food. Repeat only when your team authorises the next cycle.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | First month | Later cycles | A hold or procedure | After treatment |
|---|---|---|---|---|---|
| **Blood pressure** NCCP 00890 | Control existing hypertension | Weekly blood-pressure checks | Monthly and as clinically indicated | Review control before restarting | Follow persistent hypertension as agreed |
| **Blood count, kidneys, liver and thyroid** NCCP 00890 | Baseline tests | Regular tests on the team plan | Continue regular tests | Repeat abnormal tests as directed | Follow unresolved effects |
| **Urine protein** NCCP 00890 | Baseline urine dipstick | Regular urine checks | Extra measurement if protein is detected | Review kidney/urine findings before restarting | Individual follow-up if abnormal |
| **INR if taking warfarin** NCCP 00890 | Review anticoagulation | Weekly INR until a stable dose | Then before each cycle | Additional checks when medicines or health change | Anticoagulation-team plan |
| **Scans and clinical review** NCCP 00890 | Confirm previous treatments and goals | Review symptoms, skin, nutrition and adherence | Disease assessment as directed by consultant | Team authorises any restart | Agree subsequent care and contacts |

## What each check is for

1

**21 capsule days, then seven rest days**

28-day cycle

Fruquintinib is a targeted capsule treatment to help control bowel cancer that has spread after previous treatments. The team checks which treatments you have received, whether they remain suitable, and your priorities.

**Ask / act:** Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00890

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.35

YOUR CYCLE, DAY BY DAY

# Your 28-day cycle.

Select a day to explore what to watch for. These are possible patterns, not a promise about when symptoms start or stop.

> The full information is available below. Interactive day, phase and date controls require JavaScript; use “Read the whole cycle as text” for the complete timeline.

Your date stays on this page only. It is not saved or added to a QR code.

**Medicines on Day 1:** Fruquintinib

On a small screen, swipe the chart sideways. Each day is a keyboard-accessible button.




The calendar shows capsule days and rest days. Symptoms are not confined to a particular week.

**DAY 1**

### Start with a clear calendar

Take the prescribed daily dose with or without food. Keep blood-pressure appointments and check hands and feet.

**Your next step:** Confirm the first tests, urine checks and skin-care instructions.

The next cycle starts 28 days after this one, if your team confirms it.

> Use your oncology unit's urgent contact plan throughout the cycle, including tablet-free days. Symptoms can need attention before the next blood test or appointment.

**Read the whole cycle as text**

### Day 1–7: Start with a clear calendar

Take the prescribed daily dose with or without food. Keep blood-pressure appointments and check hands and feet. Confirm the first tests, urine checks and skin-care instructions.

### Day 8–21: Continue capsules and monitoring

Symptoms may develop over time. Report painful hands/feet, bowel changes, poor appetite, bleeding or fatigue. Blood pressure is checked weekly during the first month. Do not wait until the rest week to report problems.

### Day 22–28: Capsule-free week

Take no fruquintinib. Symptoms may persist. The team may delay the next cycle or prescribe a lower dose. Complete planned checks and confirm the next dose and start date.

Sources: nccp

PRACTICAL TREATMENT GUIDE

## Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

### Prescription and calendar

Take the prescribed fruquintinib dose once daily on Days 1–21, then take no fruquintinib on Days 22–28. Take it at about the same time each day, with or without food. Repeat only when your team authorises the next cycle.



STEP 2

### Taking capsules

The usual starting dose is 5 mg once daily, but follow your own label. Swallow capsules whole with water, with or without food; do not chew, dissolve or open them. Tell the team before starting if swallowing capsules is difficult.



STEP 3

### Missed or vomited dose

If a dose is less than 12 hours late, take it and then follow the usual schedule. If more than 12 hours late, skip it and take the next dose at the normal time. At exactly 12 hours, or if unsure, ask your pharmacist or team. Do not take a replacement capsule after vomiting; take the next dose at the usual time.

STEP 4

### Tests and visits

Baseline and regular blood count, kidney/liver profile, thyroid function and urine protein. Blood pressure must be controlled before starting, then checked weekly for the first month and monthly thereafter, or more often if needed. For warfarin, INR weekly until stable, then before each cycle. Scans and markers follow the consultant plan.

STEP 5

### Storage

Keep the original bottle tightly closed to protect from moisture and out of children's sight and reach. No special temperature conditions are required. Follow the expiry date and ask the pharmacist how to return unused capsules; do not put them into household waste or wastewater.

STEP 6

### Treatment changes

Your team must confirm dose changes and restarts. Bring the capsule pack, current calendar and medicine list to review.

Open practical treatment guide · Printable practical guide

Sources: NCCP 00890 v2a official fruquintinib regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

# Support around the medicines

### 1 Skin care

Use the unit’s skin-care plan and report redness, pain, swelling or blisters early. The team may prescribe creams or change treatment. Do not wait until walking or everyday hand use is difficult.

### 2 Sickness and bowels

This oral medicine has minimal-to-low emetogenic potential. Routine premedication is not usually needed; agree any anti-sickness and diarrhoea medicines with the team. Persistent vomiting, diarrhoea, poor intake or reduced urine needs prompt advice.

### 3 Blood pressure and medication review

Take prescribed blood-pressure medicines and keep monitoring visits. Tell the team about anticoagulants and every prescription, over-the-counter and herbal medicine.

### 4 Pregnancy and breastfeeding

Use highly effective contraception during treatment and for at least two weeks after the last dose if you could become pregnant. Do not breastfeed during treatment or for at least two weeks afterwards.

Sources: nccp · smpc

TREATMENT REFERENCE · 01

# Why this treatment may be offered

Confirm the treatment history and personal goals.

### Previously treated bowel cancer

Fruquintinib is a targeted capsule treatment to help control bowel cancer that has spread after previous treatments. The team checks which treatments you have received, whether they remain suitable, and your priorities. The team confirms the full treatment history, including chemotherapy, anti-VEGF and anti-EGFR treatment where applicable, and progression or intolerance on trifluridine/tipiracil or regorafenib.

Sources: nccp · smpc

### Evidence and uncertainty

Median overall survival was 7.4 months with fruquintinib and 4.8 months with placebo. These group medians are not a prediction of one person’s lifespan or a guaranteed extra 2.6 months. The team should discuss the balance of benefit and side effects for you.

Sources: fresco2

### Treatment duration

There is no fixed total number of cycles. Continue while benefit and tolerability justify it; the team reviews symptoms, scans, tests and your preferences.

Sources: nccp · smpc

TREATMENT REFERENCE · 02

# How fruquintinib works

It affects signals involved in blood-vessel growth.

### VEGF receptor signals

Fruquintinib blocks VEGF receptor signals that help tumours form blood vessels. Reducing these signals can slow cancer growth. Blood vessels in healthy tissues are also affected, which helps explain blood-pressure, bleeding, protein-in-urine and wound-healing risks.

Sources: nccp · smpc

### Safety checks

Blood-pressure and urine-protein changes can develop without symptoms. Tests and early symptom reporting help the team decide whether treatment can safely continue.

Sources: nccp · smpc

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TREATMENT REFERENCE · 03

## Your capsule and rest-day plan

Use your own prescription and calendar.

### Days 1–21 and Days 22–28

Take the prescribed fruquintinib dose once daily on Days 1–21, then take no fruquintinib on Days 22–28. Take it at about the same time each day, with or without food. Repeat only when your team authorises the next cycle.

Sources: nccp · smpc

**Swallow whole, with or without food**

The usual starting dose is 5 mg once daily, but follow your own label. Swallow capsules whole with water, with or without food; do not chew, dissolve or open them. Tell the team before starting if swallowing capsules is difficult.

Sources: nccp · smpc

**Missed or vomited dose**

If a dose is less than 12 hours late, take it and then follow the usual schedule. If more than 12 hours late, skip it and take the next dose at the normal time. At exactly 12 hours, or if unsure, ask your pharmacist or team. Do not take a replacement capsule after vomiting; take the next dose at the usual time.

Sources: nccp · smpc

**If too much is taken**

Contact your doctor immediately if you take more than prescribed. You may need medical attention; take the pack and treatment calendar with you.

Sources: nccp · smpc

TREATMENT REFERENCE · 04

# Checks and follow-up

Feeling well does not replace testing.

### Before and during treatment

Baseline and regular blood count, kidney/liver profile, thyroid function and urine protein. Blood pressure must be controlled before starting, then checked weekly for the first month and monthly thereafter, or more often if needed. For warfarin, INR weekly until stable, then before each cycle. Scans and markers follow the consultant plan.

Sources: nccp · smpc

### If a test is abnormal

The team may arrange extra blood or urine tests, treat hypertension, pause capsules, reduce the dose or stop treatment. Do not change the dose yourself.

Sources: nccp · smpc

### Scans and tumour markers

The consultant decides the imaging and marker schedule for your situation. This regimen does not mandate a fixed CT or CEA interval.

Sources: nccp · smpc

### After a pause or stopping

Ask who follows persistent symptoms and abnormal tests, and when the next review is due. Do not restart without clear instructions.

Sources: nccp · smpc

TREATMENT REFERENCE · 05

## Skin, mouth, eating and energy

Early support can help preserve daily activities.

### Hands and feet

Use the unit’s skin-care plan and report redness, pain, swelling or blisters early. The team may prescribe creams or change treatment. Do not wait until walking or everyday hand use is difficult.

Sources: nccp · smpc

### Bowels, fluids and sickness

This oral medicine has minimal-to-low emetogenic potential. Routine premedication is not usually needed; agree any anti-sickness and diarrhoea medicines with the team. Persistent vomiting, diarrhoea, poor intake or reduced urine needs prompt advice.

Sources: nccp · smpc

### Mouth and appetite

Report mouth soreness, voice changes, poor appetite or weight loss. Ask for mouth care and nutritional support. Drink as advised for your health.

Sources: nccp · smpc

### Fatigue and thyroid

New marked fatigue, feeling unusually cold, weight or skin changes may need thyroid assessment. Balance activity with rest and report symptoms affecting usual activities.

Sources: nccp · smpc

### Driving

Do not drive or use machinery if tiredness or other symptoms affect concentration or reactions.

Sources: smpc

TREATMENT REFERENCE · 06

# Symptoms needing urgent help

A rare effect may still need immediate care.

### Bleeding and bowel problems

Seek urgent help for black or bloody stools, blood in urine, coughing/vomiting blood, severe or persistent abdominal pain. Heavy bleeding or collapse needs emergency care.

Sources: nccp · smpc

### Neurological, heart or vascular symptoms

Severe headache with confusion, seizure or visual changes, chest pain, severe breathlessness, sudden weakness or sudden severe chest/back/abdominal pain requires emergency help.

Sources: nccp · smpc

### Kidneys, skin and infection

Report much less urine or swelling, painful hands/feet, fever, shivering or suddenly feeling unwell. Symptoms may need assessment even during the capsule-free week.

Sources: nccp · smpc

TREATMENT REFERENCE · 07

# Other medicines, wounds and procedures

Tell every treating team you take fruquintinib.

### Medication review

Tell the pharmacist about anticoagulants, antibiotics, infection treatments, seizure medicines, herbal remedies and supplements. Rifampicin and other strong enzyme inducers can affect fruquintinib exposure. Ask before starting or stopping anything.

Sources: smpc

### Wound healing and surgery

Report unhealed wounds and planned surgery. Product information recommends withholding fruquintinib for at least two weeks before surgery and not restarting for at least two weeks afterwards and until adequate wound healing. Your oncology and surgical teams must agree the exact plan; do not stop or restart it yourself.

Sources: smpc

### Allergies

Tell the pharmacist about medicine or colouring-agent allergies. Capsule colours differ by strength and contain colouring agents that can cause allergic reactions.

Sources: smpc

TREATMENT REFERENCE · 08

# Pregnancy precautions and safe storage

Keep the medicine in its labelled packaging.

### Pregnancy and feeding

Use highly effective contraception during treatment and for at least two weeks after the last dose if you could become pregnant. Do not breastfeed during treatment or for at least two weeks afterwards.

Sources: nccp · smpc

### Storage

Keep the original bottle tightly closed to protect from moisture and out of children's sight and reach. No special temperature conditions are required. Follow the expiry date and ask the pharmacist how to return unused capsules; do not put them into household waste or wastewater.

Sources: smpc

### Your treatment record

Carry the wallet card and current medicine list. Record the prescribed dose, any holds, symptoms, BP and upcoming tests. Replace all example telephone numbers with your real oncology contacts before using the card.

Sources: nccp · smpc

## Evidence in previously treated colorectal cancer

FRESCO-2 compared fruquintinib plus best supportive care with placebo plus best supportive care after previous metastatic colorectal treatments.

Median overall survival was 7.4 months with fruquintinib and 4.8 months with placebo. These group medians are not a prediction of one person's lifespan or a guaranteed extra 2.6 months. The team should discuss the balance of benefit and side effects for you.

Sources: fresco2 · smpc

PREPARE FOR THE CONVERSATION

# Questions worth asking

**undefined**

undefined

**undefined**

undefined

**undefined**

undefined

**undefined**

undefined

**undefined**

undefined

**undefined**

undefined

**Try explaining the plan back:** Can you show the 21 tablet days and seven rest days, explain the meal and missed-dose instructions, identify the early liver and blood-pressure checks, and say whom you will call if unwell?

SOURCE TRANSPARENCY

# Where this information comes from

Regimen schedule: NCCP 00890 v2a. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00890 v2a official fruquintinib regimen
- EMA Fruzaqla product information and patient leaflet
- FRESCO-2 randomised phase III trial
- HSE treatment safety and urgent advice

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SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-05.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

## Written source addresses

https://healthservice.hse.ie/documents/6648/Fruquintinib_890.pdf

https://www.ema.europa.eu/en/documents/product-information/fruzaqla-epar-product-information_en.pdf

https://pubmed.ncbi.nlm.nih.gov/37331369/

https://www2.hse.ie/conditions/chemotherapy/side-effects/

BASELINE → TREATMENT → FOLLOW-UP

# Tests before, during and after treatment

NCCP specifies regular blood/urine/thyroid tests but no universal frequency for each. Ask your team to write the dates. Scans and tumour markers are individualised.

| Check | Before starting | First month | Later cycles | A hold or procedure | After treatment |
|---|---|---|---|---|---|
| **Blood pressure** | Control existing hypertension | Weekly blood-pressure checks | Monthly and as clinically indicated | Review control before restarting | Follow persistent hypertension as agreed |
| **Blood count, kidneys, liver and thyroid** | Baseline tests | Regular tests on the team plan | Continue regular tests | Repeat abnormal tests as directed | Follow unresolved effects |
| **Urine protein** | Baseline urine dipstick | Regular urine checks | Extra measurement if protein is detected | Review kidney/urine findings before restarting | Individual follow-up if abnormal |
| **INR if taking warfarin** | Review anticoagulation | Weekly INR until a stable dose | Then before each cycle | Additional checks when medicines or health change | Anticoagulation-team plan |
| **Scans and clinical review** | Confirm previous treatments and goals | Review symptoms, skin, nutrition and adherence | Disease assessment as directed by consultant | Team authorises any restart | Agree subsequent care and contacts |

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00890 v2a official fruquintinib regimen

THE WHOLE TREATMENT JOURNEY

# Fruquintinib (Fruzaqla) monotherapy: treatment and checks

**Metastatic colorectal cancer after standard therapies and trifluridine/tipiracil or regorafenib**

Take the prescribed fruquintinib dose once daily on Days 1–21, then take no fruquintinib on Days 22–28. Take it at about the same time each day, with or without food. Repeat only when your team authorises the next cycle.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | First month | Later cycles | A hold or procedure | After treatment |
|---|---|---|---|---|---|
| **Blood pressure** NCCP 00890 | Control existing hypertension | Weekly blood-pressure checks | Monthly and as clinically indicated | Review control before restarting | Follow persistent hypertension as agreed |
| **Blood count, kidneys, liver and thyroid** NCCP 00890 | Baseline tests | Regular tests on the team plan | Continue regular tests | Repeat abnormal tests as directed | Follow unresolved effects |
| **Urine protein** NCCP 00890 | Baseline urine dipstick | Regular urine checks | Extra measurement if protein is detected | Review kidney/urine findings before restarting | Individual follow-up if abnormal |
| **INR if taking warfarin** NCCP 00890 | Review anticoagulation | Weekly INR until a stable dose | Then before each cycle | Additional checks when medicines or health change | Anticoagulation-team plan |
| **Scans and clinical review** NCCP 00890 | Confirm previous treatments and goals | Review symptoms, skin, nutrition and adherence | Disease assessment as directed by consultant | Team authorises any restart | Agree subsequent care and contacts |

# What each check is for

1

### 21 capsule days, then seven rest days

28-day cycle

Fruquintinib is a targeted capsule treatment to help control bowel cancer that has spread after previous treatments. The team checks which treatments you have received, whether they remain suitable, and your priorities.

**Ask / act:** Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00890

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.35

# Fruquintinib (Fruzaqla) monotherapy

**Previously treated colorectal cancer** The team confirms the full treatment history, including chemotherapy, anti-VEGF and anti-EGFR treatment where applicable, and progression or intolerance on trifluridine/tipiracil or regorafenib.

**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Separate scripts for each phase

Choose the active phase in the clinician workflow. Each script has its own medicines, timing and review checks.

- **Fruquintinib: Days 1–21, then Days 22–28 off** — Minimal-to-low oral Fruquintinib

https://sactcheck.com/patient/00890/supportive-care.html

# Fruquintinib (Fruzaqla) monotherapy

**Previously treated colorectal cancer** The team confirms the full treatment history, including chemotherapy, anti-VEGF and anti-EGFR treatment where applicable, and progression or intolerance on trifluridine/tipiracil or regorafenib.

**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Fruquintinib: Days 1–21, then Days 22–28 off

**Active medicines:** Fruquintinib

**Emetogenic risk:** Minimal-to-low oral

| Medicine | Dose / route | When |
|---|---|---|
| Rescue antiemetic | Individual prescription under the oral-SACT pathway | As required; no routine daily prophylaxis<br>NCCP classification V6 Table 5 provides metoclopramide, prochlorperazine or one 5-HT3 antagonist as options; choose rather than automatically combine, after interaction/ contraindication review. |

### Additional phase-specific support

- Premedication is not usually required. Use local skin-care guidance for hand-foot skin reactions and report painful, blistered or function-limiting symptoms early.
- Blood pressure weekly for the first month, then monthly and as clinically indicated. Keep prescribed antihypertensive treatment and report severe headache, visual change, chest pain or neurological symptoms urgently.
- Baseline and regular FBC, renal/liver profile, urine dipstick for protein and thyroid function. INR if clinically indicated; if taking warfarin, weekly INR until a stable warfarin dose is established, then before each cycle.
- Review bleeding, gastrointestinal perforation, wounds and planned surgery; follow the separately prescribed interruption/ restart plan. Do not borrow the regorafenib dose-modification table.
- Women of childbearing potential: highly effective contraception during treatment and for at least two weeks after the last dose, per NCCP.
- Confirm the prescribed daily dose and Days 1–21 calendar; dose reductions are consultant-led and treatment must stop if 3 mg daily is not tolerated.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00890 v2a · p2,4–5 · NCCP regimen; NCIS Medical Oncology antiemetics V8 Table 1

Sources checked 2026-11-05 · SACTCheck v0.84.35 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

# Fruquintinib (Fruzaqla) monotherapy

NCCP 00890 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

## Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

### Prescription and calendar

Take the prescribed fruquintinib dose once daily on Days 1–21, then take no fruquintinib on Days 22–28. Take it at about the same time each day, with or without food. Repeat only when your team authorises the next cycle.

STEP 2

### Taking capsules

The usual starting dose is 5 mg once daily, but follow your own label. Swallow capsules whole with water, with or without food; do not chew, dissolve or open them. Tell the team before starting if swallowing capsules is difficult.

STEP 3

### Missed or vomited dose

If a dose is less than 12 hours late, take it and then follow the usual schedule. If more than 12 hours late, skip it and take the next dose at the normal time. At exactly 12 hours, or if unsure, ask your pharmacist or team. Do not take a replacement capsule after vomiting; take the next dose at the usual time.

STEP 4

### Tests and visits

Baseline and regular blood count, kidney/liver profile, thyroid function and urine protein. Blood pressure must be controlled before starting, then checked weekly for the first month and monthly thereafter, or more often if needed. For warfarin, INR weekly until stable, then before each cycle. Scans and markers follow the consultant plan.

STEP 5

### Storage

Keep the original bottle tightly closed to protect from moisture and out of children’s sight and reach. No special temperature conditions are required. Follow the expiry date and ask the pharmacist how to return unused capsules; do not put them into household waste or wastewater.

STEP 6

### Treatment changes

Your team must confirm dose changes and restarts. Bring the capsule pack, current calendar and medicine list to review.

Sources: NCCP 00890 v2a official fruquintinib regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

# Fruquintinib (Fruzaqla) monotherapy

## A record of what you experience

One cycle per page. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal CTCAE grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Take the prescribed fruquintinib dose once daily on Days 1–21, then take no fruquintinib on Days 22–28. Take it at about the same time each day, with or without food. Repeat only when your team authorises the next cycle.

The four pages are reusable recording sheets, not a prescribed four-cycle course. Ask for additional sheets as needed.

**Emergency: 112 / 999.** Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

**Contact oncology immediately** for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

**Insert numbers here**

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

**Insert numbers here**

Ask for medical oncology

EVENINGS / OVERNIGHT

**Your out-of-hours pathway / nearest ED**

Other hospitals: use the numbers your team provides.

Your own hospital / contact instructions:

https://sactcheck.com/patient/00890/

Full patient information, sources and visual timeline. No identifiers are encoded.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

# Fruquintinib (Fruzaqla) monotherapy

Cycle number: _____ Start date: _____ Next review: _____

Record the prescribed dose, treatment days, rest days, blood-pressure readings and tests. A diary entry is not permission to restart after a hold.

### Fruquintinib · Targeted therapy capsule

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Hand-foot skin reaction** | ☐ None | ☐ Redness or tingling without pain | ☐ Pain limits usual tasks: call | ☐ Pain limits washing / dressing: urgent review | ☐ Acute severe illness: emergency help |
| **Diarrhoea** | ☐ Usual pattern | ☐ Up to 3 extra stools above normal | ☐ 4–6 extra stools/day or at night: call | ☐ 7+ extra stools/day or dehydration: urgent review | ☐ Collapse / severe dehydration: 112/999 |
| **Mouth soreness / ulcers** | ☐ None | ☐ Mild soreness; eating normally | ☐ Painful; soft food needed: call | ☐ Cannot eat / drink enough: urgent review | ☐ Major breathing difficulty: 112/999 |
| **Fatigue** | ☐ Usual energy | ☐ Tiredness improves with rest | ☐ Rest does not help; usual tasks affected: call | ☐ Self-care difficult: urgent review | ☐ Collapse: 112/999 |

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

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# Keep your treatment details to hand

**Metastatic colorectal cancer after standard therapies and trifluridine/tipiracil or regorafenib** · NCCP 00890 v2a · Document revised 5 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

**FRONT / CONTACTS**

## My Treatment Card

SACTCheck

Name: _____

**Fruquintinib (Fruzaqla)**

28 days · capsules Days 1–21 · no capsules Days 22–28.

Phase: _____ Cycle plan: team to confirm

**Daytime (EXAMPLE)** 087 12345678

**Out of hours (EXAMPLE)** 087 87654321

**Unwell or fever? Call oncology now.**
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

CUT outer border

**BACK / TREATMENT RECORD**

## My cycle record

Rolling record — follow your agreed treatment plan.

| Cycle | Date / initials |
|---|---|
| | |
| | |
| | |
| | |

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

**FOLD centre — do not cut**

## Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

## Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

### iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

### Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

## Your local contacts and resources

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

Hospital / team: _____
Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.35 · NCCP 00890 v2a · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.